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2026 Ebola Outbreak: Frequently Asked Questions

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2026 Ebola Outbreak: Frequently Asked Questions

First reported in May 2026, the current Ebola disease outbreak, caused by Bundibugyo virus and concentrated in northeastern Democratic Republic of Congo (DRC), has rapidly become the second-largest Ebola outbreak on record. According to the U.S. Centers for Disease Control and Prevention (CDC), the outbreak has expanded “substantially faster than previous Ebola outbreaks.” The United States has responded to the outbreak with actions taken at home and abroad, including selected domestic preparedness measures, travel restrictions, foreign assistance, and investments in certain vaccine and therapeutics trials, among other actions. As of October 6, no Ebola cases associated with this outbreak have been identified in the United States.

Members of Congress have issued statements related to the outbreak, conducted oversight of the Administration’s response, and introduced legislation. This CRS Frequently Asked Questions report provides information related to the virus and its spread, the current outbreak, and U.S. and international responses, among other issues. It may be updated as the situation evolves.

SUMMARY

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Contents

Introduction	1
Frequently Asked Questions	4
What is Ebola disease and how does it spread?	4
Where is the current outbreak located?	5
What are the primary challenges to outbreak control in the DRC?.....	6
How has the United States responded to the outbreak at home?.....	8
U.S. Domestic Preparedness for Ebola Introduction	8
U.S. Travel Restrictions	9
How has the United States responded to the outbreak overseas?.....	12
U.S. Foreign Assistance	12
Kenya Quarantine Facility	14
How is the United States funding its international response?	14
How is the outbreak affecting other U.S. foreign policy priorities in Central Africa?.....	15
How has the World Health Organization (WHO) responded to the outbreak? Has the United States engaged with WHO?.....	17
How, if at all, are other aid donors responding to the crisis?	18
What is the current status of vaccines and therapeutics? How is the United States participating in such efforts?	19
Vaccines	19
Therapeutics	21

Figures

Figure 1. Known Ebola Outbreaks: History	2
Figure 2. Map: Ebola Outbreaks in DRC to Date.....	6

Contacts

Author Information.....	22
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Introduction

The current Ebola disease outbreak, concentrated in northeastern Democratic Republic of Congo (DRC), has rapidly become the second largest on record, surpassing a 2018-2020 DRC outbreak and possibly on track to surpass the 2014-2016 West Africa Ebola outbreak, the largest ever.¹ This outbreak has expanded “substantially faster than previous Ebola outbreaks,” according to the U.S. Centers for Disease Control and Prevention (CDC).² It was first reported in May 2026, after apparently spreading unidentified for months.³ As of October 6, no Ebola cases associated with this outbreak have been identified in the United States. The State Department, which is leading the U.S. international response to the outbreak, has identified three “priority pillars” of the response: (1) “protect the homeland and Americans at home and abroad,” (2) “support countries to contain the outbreak at its source,” and (3) “mobilize global will to end the outbreak.”⁴

The outbreak is unfolding in a region already staggering under the weight of conflicts, mass displacement, overlapping public health challenges, and widespread poverty. So far, the outbreak has killed and sickened thousands of people, most of them in DRC. Experts assess that the disease could spread to neighboring countries and last a year or longer.⁵ The 2018-2020 Ebola outbreak, in the same vicinity in northeastern DRC, took two years to contain.⁶ The current outbreak may have an enduring impact on already fragile economies and health systems in Central Africa.⁷

There have been dozens of Ebola outbreaks in Africa since the disease was first identified in DRC in 1976 (**Figure 1**). The current outbreak is caused by the relatively rare Bundibugyo virus, which diagnostic tests initially failed to detect.⁸ There is no approved vaccine or treatment specifically for Bundibugyo, although efforts are underway to develop and test candidates (as discussed below). Prior to the 2014-2016 West Africa outbreak, Ebola outbreaks were contained without such tools. Vaccines and antiviral treatments for the more common Zaire ebolavirus were developed during that outbreak and were used to contain the 2018-2020 DRC outbreak, also caused by Zaire.⁹ Research and development (R&D) is underway to identify vaccines and therapeutics effective against Bundibugyo, including the extent to which such tools approved for use against Zaire may be deployed or modified for Bundibugyo.

¹ For case and fatality data, see U.S. Centers for Disease Control and Prevention (CDC), “Ebola Outbreak: Current Situation” at <https://www.cdc.gov/ebola/situation-summary/index.html>. The 2024-2016 West Africa Ebola outbreak infected over 28,600 people and caused over 11,300 deaths; see CDC, “History of Ebola Outbreaks.”

² CDC, “Ebola Outbreak: Current Situation,” as of September 29, 2026.

³ CDC modeling has estimated that the outbreak likely started around mid- to late February 2026. CDC, “Update on Ebola Outbreak in the Democratic Republic of the Congo and Uganda,” June 5, 2026.

⁴ Department of State, “State Department Ebola Response Updates,” at <https://www.state.gov/state-department-ebola-response-updates>, accessed September 30, 2026.

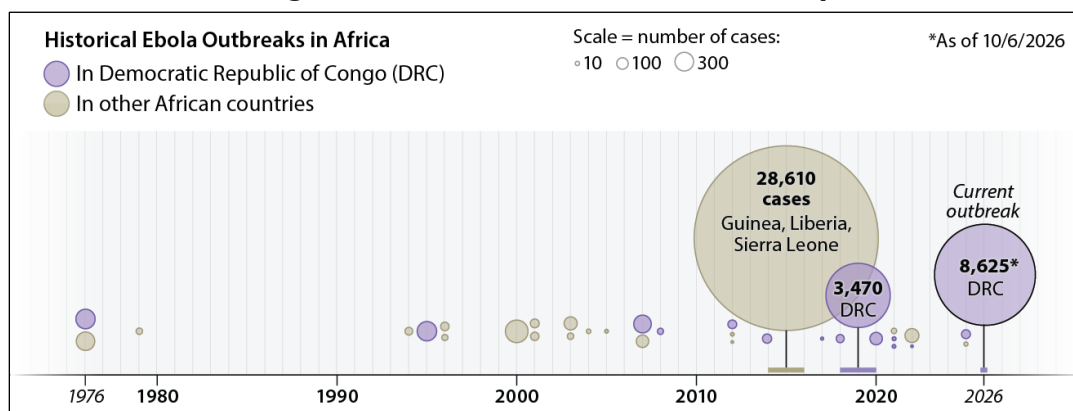
⁵ *New York Times*, “Ebola Outbreak Could Become Worst on Record, Africa C.D.C. Chief Warns,” June 16, 2026.

⁶ See CRS Report R45933, *Ebola Outbreaks in the Democratic Republic of Congo: Emergencies or Enduring Threat?*

⁷ Bloomberg, “Ebola Is Hollowing Out Basic Health Care as Congo Outbreak Grows,” September 29, 2026.

⁸ Krutika Kuppalli et al., “A review of Bundibugyo virus and the 2026 outbreak: lessons for epidemic preparedness,” *The Lancet*, July 31, 2026.

⁹ See, for example, London School of Hygiene & Tropical Medicine, “Ebola vaccine provides long-term immune responses against deadly virus, helping inform future outbreak responses,” April 29, 2026; William A. Fischer and David A. Wohl, “Combining vaccines, optimised supportive care, and therapeutics for Ebola virus disease increases survival,” *The Lancet*, vol. 24, 6 (June 2024); L’Emir Wassim El Ayoubi et al., “Recent advances in the treatment of Ebola disease: A brief overview,” *PLOS*, March 15, 2024; and Ewen Callaway, “‘Make Ebola a thing of the past’: first vaccine against deadly virus approved,” *Nature*, November 12, 2019.

Figure 1. Known Ebola Outbreaks: History

Source: CRS graphic, based on CDC, “History of Ebola Outbreaks,” as of October 6, 2026.

The current outbreak and response raise a number of potential issues for Congress. These include the possible authorization and appropriation of funds for U.S. personnel, programs, and partners seeking to contain the outbreak and prevent (or respond to) cases in the United States, should they develop. Congress may also conduct oversight of program implementation, interagency coordination, and executive branch actions more broadly.¹⁰ Multiple U.S. federal entities are involved in response and prevention efforts, including the State Department, the CDC, the U.S. Biomedical Advanced Research and Development Authority (BARDA), the Department of Defense,¹¹ and Customs and Border Protection (CBP)—with implications for congressional committee jurisdictions and oversight options.

Members of Congress may consider a range of issues as the outbreak and international responses evolve. Congress shaped U.S. responses to past Ebola outbreaks in various ways. For example, in late 2014 during the West Africa outbreak, after two Ebola-infected individuals separately entered the United States (and one transmitted the virus to two U.S. nurses, the first ever Ebola transmission on U.S. soil), some Members of Congress called on the Obama Administration to restrict the entry of travelers from Ebola-affected countries in West Africa.¹² Soon after, the Administration established a system to screen and follow up on all such travelers, although it did not impose a full travel ban.¹³ As the West Africa outbreak escalated, Congress enacted more than \$5.4 billion in emergency funds for domestic and global efforts to contain the virus.¹⁴ Congress also made funds available for Ebola prevention and response under regular appropriations measures during the 2018-2020 DRC outbreak.¹⁵ (U.S. Ebola preparedness and response efforts,

¹⁰ The U.S. Agency for International Development Office of Inspector General (USAID OIG) has highlighted “insights and lessons” from its audits and investigations of U.S. responses to past Ebola outbreaks. USAID OIG, “Oversight of U.S. Government Ebola Responses and Emerging Global Health Threats,” June 24, 2026.

¹¹ The Department of Defense is using “Department of War” as a secondary designation per Executive Order 14347, dated September 5, 2025.

¹² CNN, “Congressmen to Obama: Step up Ebola defense,” October 9, 2014.

¹³ CDC, “Overview, Control Strategies, and Lessons Learned in the CDC Response to the 2014–2016 Ebola Epidemic,” July 8, 2016, and *Supplement: Travel and Border Health Measures to Prevent the International Spread of Ebola*; and CRS Report R43809, *Preventing the Introduction and Spread of Ebola in the United States: Frequently Asked Questions* [2014].

¹⁴ CRS Report R43807, *FY2015 Funding to Counter Ebola and the Islamic State (IS)*. Because Congress provided such funding with an emergency designation, it was not subject to congressionally established discretionary budget caps.

¹⁵ For example, P.L. 116-94 (Further Consolidated Appropriations Act, 2020) made \$85 million available for deposit to (continued...)

including during the current outbreak, have otherwise been funded with existing, non-Ebola-specific resources.)

Lessons learned from congressional oversight might also inform Members as they seek to shape the future of foreign assistance.¹⁶ The current Ebola outbreak is the first declared public health emergency of international concern (PHEIC) to emerge since the Trump Administration undertook major changes to the U.S. foreign assistance structure in 2025.¹⁷ Such changes include the dismantlement of the U.S. Agency for International Development (USAID), termination of numerous foreign assistance programs, and reductions-in-force at the Department of State and the CDC, among others. The Administration has also withdrawn the United States from the World Health Organization (WHO), with which previous Administrations worked to coordinate Ebola responses.¹⁸ Observers have discussed how, if at all, such changes have affected the U.S. response to the crisis.¹⁹ The State Department has highlighted the U.S. role as the leading donor to the response, its rapid activation of a “dedicated Ebola Response Task Force,” and what it has termed the “fastest-ever” deployment of an Ebola-focused U.S. Disaster Assistance Response Team” to coordinate response efforts in DRC and Uganda.²⁰ Challenges inherent in identifying the Bundibugyo virus, preparedness capacity in the region, and other factors likely inhibited initial detection and response efforts in any case.²¹

CDC’s Infectious Diseases Rapid Response Reserve Fund for potential Ebola preparedness and response activities, and made \$535 million available to the Department of Health and Human Services’ Public Health and Social Services Emergency Fund for purchase of vaccines, therapeutics, and diagnostics for the prevention and treatment of Ebola. The act further provided that bilateral economic and security assistance “shall be made available for assistance for the Democratic Republic of the Congo for stabilization, global health, and bilateral economic assistance, including in areas affected by, and at risk from, the Ebola virus disease.”

¹⁶ Selected analysts and practitioners have sought to determine initial lessons learned and apply them to future foreign assistance policy. For example, Norma Ojehomon et al., “How the Current Ebola Outbreak Might Reshape American Foreign Aid,” *Health Affairs Forefront*, August 26, 2026.

¹⁷ A public health emergency of international concern (PHEIC) is declared under the International Health Regulations (IHR). For more information, see “How has the World Health Organization (WHO) responded to the outbreak? Has the United States engaged with WHO?”

¹⁸ Executive Order 14155, “Withdrawing the United States From the World Health Organization,” 90 *Federal Register* 8361, January 20, 2025.

¹⁹ See, for example, Oisín Downes, *Cuts to USAID and the Ebola Outbreak in the DRC*, The Borgen Project, June 14, 2026; Stephanie Psaki, Anya Hirschfeld, and Allison Krugman, *Charting Ebola Responses: How 2026 Stacks Up After Aid Cuts*, Think Global Health, June 1, 2026; KFF, “Comparing U.S. Ebola Outbreak Response Capabilities and Practices Over Time,” July 28, 2026; CNBC, “Ebola outbreak containment hampered by USAID closure, experts say,” July 6, 2026; and Maya Brownstein, “Ebola’s spread fueled by cuts in humanitarian aid [interview with Associated Professor Phuong Pham],” Harvard T.H. Chan School of Public Health, June 18, 2026.

²⁰ Department of State, “Ebola Response Update,” May 23, 2026; see also “State Department Ebola Response Updates,” <https://www.state.gov/state-department-ebola-response-updates>.

²¹ Thirumalaisamy P. Velavan, “Bundibugyo Ebola: why preparedness still fails at the point of detection,” *The Lancet*, July 2026.

Frequently Asked Questions

What is Ebola disease and how does it spread?

Ebola disease is caused by a genus of viruses known as *orthoebolaviruses* (also commonly referred to *ebolaviruses*).²² The current outbreak is caused by the Bundibugyo virus within this genus, which to date has been found in DRC and Uganda.²³

Ebola disease is severe and often fatal in humans.²⁴ It is associated with a viral hemorrhagic fever, which typically begins with symptoms such as fever, fatigue, and pain that progress to symptoms such as vomiting, diarrhea, and impaired kidney and liver functions.²⁵ Internal and external bleeding are less frequent symptoms that can occur at a late stage in the disease.²⁶ The average case fatality rate (death rate among confirmed cases) from Ebola is around 50%, but such rates have varied across different Ebola outbreaks, ranging from 25%-90%.²⁷ As of September 25, 2026, the crude case fatality rate in the 2026 Bundibugyo virus outbreak was calculated at 48% (confirmed deaths divided by confirmed cases).²⁸ Given that some individuals with Ebola may not seek health care, the crude case fatality may differ from the true rate.²⁹

Ebola disease spreads from person to person through direct contact with blood or other bodily fluids of an infected person or through contact with surfaces or materials contaminated with these fluids. Disease onset typically does not begin until two to 21 days after exposure (incubation period), and people are not infectious until they show symptoms.³⁰ Within communities affected by the outbreak, caregivers, health care workers, and people handling those who have died from Ebola and their remains are most at risk of contracting Ebola.³¹

Ebola outbreaks and cases have been reported periodically in African countries since 1976 (see **Figure 1**, above).³² Smaller outbreaks of Bundibugyo virus occurred in the DRC and Uganda in 2007-2008 and in 2012.³³ Ebola outbreaks have originated from human contact with infected animals such as bats and nonhuman primates (zoonotic spillover) and through human

²² CDC, "How Ebola Disease Spreads," accessed on September 14, 2026, <https://www.cdc.gov/ebola/causes/index.html>; and WHO, "Ebola disease," April 24, 2025.

²³ CDC, "Ebola Outbreak: Current Situation," last updated September 14, 2026, <https://www.cdc.gov/ebola/situation-summary/index.html>; and CDC, "About Viral Hemorrhagic Fevers," April 15, 2024.

²⁴ WHO, "Ebola disease," April 24, 2025.

²⁵ P.E. Rollin, "Ebola-Marburg Viral Diseases," in *Control of Communicable Diseases Manual*, ed. David L. Heymann, 21st ed. (American Public Health Association Press, 2022), p. 178; and Pierre Akilimali et al., "Clinical Characteristics of Patients Infected with Bundibugyo Virus, DRC 2026," *New England Journal of Medicine*, June 24, 2026.

²⁶ WHO, "Ebola disease," April 24, 2025.

²⁷ WHO, "Ebola disease," April 24, 2025.

²⁸ WHO, "Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo," October 8, 2026.

²⁹ It is unclear whether people with Ebola who do not seek health care have a higher or lower fatality rate than those who seek health care and are accounted for in the data.

³⁰ WHO, "Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo," October 8, 2026.

³¹ P. E. Rollin, "Ebola-Marburg Viral Diseases," in *Control of Communicable Diseases Manual*, ed. David L. Heymann, 21st ed. (American Public Health Association Press, 2022), p. 182.

³² CDC, "History of Ebola Outbreaks," as of July 25, 2026; and P. E. Rollin, "Ebola-Marburg Viral Diseases," in *Control of Communicable Diseases Manual*, ed. David L. Heymann, 21st ed. (American Public Health Association Press, 2022), p. 179-180.

³³ Joseph F. Wamala et al., "Ebola Hemorrhagic Fever Associated with Novel Virus Strain, Uganda, 2007-2008," *Emerging Infectious Diseases*, vol. 16, no. 7 (July 2010), pp. 1087-1092; and WHO, "Disease Outbreak News: 2012-Congo."

transmission, particularly where the virus has persisted at a low levels in human populations and then reemerged to cause an outbreak.³⁴ Based on analyses of the virus, the 2026 Bundibugyo outbreak appears to have originated from a zoonotic spillover event.³⁵

There are no specific vaccines or treatments for Ebola disease caused by Bundibugyo virus.³⁶ Currently available Ebola vaccines and treatments were developed to target the Zaire ebolavirus, which caused previous large-scale Ebola outbreaks in West Africa (2014-2016) and the DRC (2018-2020), and underwent clinical trials during those outbreaks.³⁷ There are ongoing efforts to test existing Zaire ebolavirus vaccines against Bundibugyo virus and to develop new specific vaccines and treatments to protect against Bundibugyo virus, as discussed in “What is the current status of vaccines and therapeutics? How is the United States participating in such efforts?”

Where is the current outbreak located?

To date, most cases in this outbreak (~76%) have been detected in DRC’s Ituri province, where the outbreak appears to have originated, although it has also spread to several other provinces, including neighboring North Kivu, where new case rates (incidence) appeared to be increasing as of late September (**Figure 2**).³⁸ WHO has assessed that the outbreak poses a “very high” risk to DRC, a “high” risk to countries bordering DRC, and a “low” risk for the rest of Africa and globally.³⁹

Neighboring Uganda has reported 21 confirmed and probable cases in the current outbreak, all reportedly among travelers from DRC or individuals who came into contact with those travelers.⁴⁰ Uganda’s last known Ebola patient was discharged from medical care on July 16, 2026, and the country was declared “Ebola free” on August 27.⁴¹ Uganda has implemented enhanced disease surveillance measures, honed from prior experience with Ebola, which likely helped with rapid case detection and isolation.⁴² One case has been identified in France (a doctor who had returned from DRC), with no further transmission there, and on October 6, 2026, Kenya confirmed its first ever case (a Kenyan citizen who had reportedly been living in DRC and traveled through Uganda before dying in Kenya’s capital, Nairobi).

No cases have been reported in the United States as of October 6, 2026. Two Americans who tested positive for Bundibugyo virus in DRC in mid-2026 were transferred to Germany for care;

³⁴ Ruth Kallay et al., “Use of Ebola Vaccines—Worldwide, 2021–2023,” *Morbidity and Mortality Weekly Report*, vol. 73, no. 16 (April 25, 2024), pp. 360–364.

³⁵ Adrienne Amuri-Aziza et al., “Emergence of a Bundibugyo virus variant in the 2026 outbreak in the Democratic Republic of the Congo and Uganda,” *Nature Medicine*, August 10, 2026.

³⁶ WHO, “Experts convened by WHO advise on candidate treatments and vaccines for Ebola disease caused by Bundibugyo virus,” press release, May 28, 2026.

³⁷ Ruth Kallay et al., “Use of Ebola Vaccines—Worldwide, 2021–2023,” *Morbidity and Mortality Weekly Report*, vol. 73, no. 16 (April 25, 2024), pp. 360–364; and Els Torreele et al., “Breakthrough treatments for Ebola virus disease, but no access—what went wrong, and how can we do better?,” *The Lancet*, vol. 23, no. 7 (July 2023), pp. e253–e258.

³⁸ WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” October 8, 2026.

³⁹ WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” October 8, 2026.

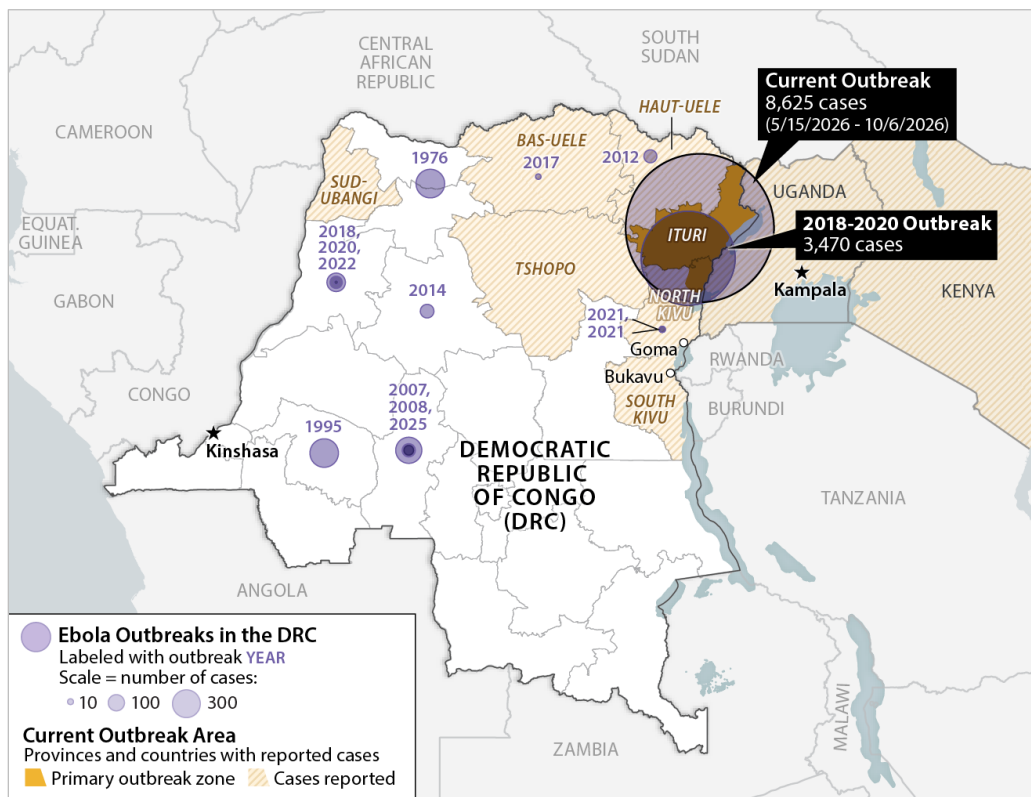
⁴⁰ CDC, “Ebola Outbreak: Current Situation,” as of October 6, 2026, and WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” June 8, 2026.

⁴¹ Countries that have experienced Ebola domestic transmission are declared “Ebola free” after two incubation periods (42 days) have passed with no new cases. WHO, “Uganda ends Ebola outbreak following completion of 42-day countdown,” August 27, 2026.

⁴² PBS, “Uganda declares itself Ebola-free as Congo continues to struggle with the outbreak,” July 29, 2026.

both recovered and were released.⁴³ The CDC assesses as of October 6 that the risk of Ebola spreading to the United States is “considered very low.”⁴⁴

Figure 2. Map: Ebola Outbreaks in DRC to Date



Source: CRS graphic, based on CDC, “History of Ebola Outbreaks,” as of October 6, 2026.

What are the primary challenges to outbreak control in the DRC?

Containment of Ebola disease outbreaks has generally involved the isolation of infected individuals; provision of personal protective equipment and training to health and burial workers and others who may come into contact with infected people; contact-tracing to ensure rapid detection and isolation of new cases; and promotion of “safe and dignified” burials. Surveillance, or collecting and analyzing data on where the disease is spreading, is also considered a key tool.⁴⁵ Successful implementation has generally involved extensive community engagement in order to educate health workers and ordinary people about how the disease spreads, encourage behavior changes, identify cases, and identify and follow up with their close contacts.⁴⁶

Limited health care infrastructure, ongoing conflicts, violence against health workers, labor unrest, population movements, and lack of trust in state institutions have inhibited contact tracing,

⁴³ Mark Osborne, “U.S. doctor with Ebola released from German hospital after recovering from deadly virus,” *CBS News*, June 6, 2026; “Ebola patient discharged from Frankfurt hospital after full recovery,” *Reuters*, July 28, 2026.

⁴⁴ CDC, “Ebola Outbreak: Current Situation,” as of October 6, 2026.

⁴⁵ Médecins Sans Frontières (MSF), “What will it take to end the Ebola disease outbreak in Democratic Republic of Congo?” September 30, 2026.

⁴⁶ See, for example, Nagashi Ngongo et al., “Community engagement should be the priority for containment of the Bundibugyo Ebola outbreak,” *Nature Health* (2026).

infection prevention and control, and treatment of patients in the current outbreak.⁴⁷ Public health experts have expressed alarm at “sustained community transmission and significant geographic expansion,” and have warned that “transmission is particularly amplified in health-care settings when [infection prevention and control] measures are inadequate and during unsafe burial practices involving direct contact with deceased individuals.”⁴⁸ Difficulties in distinguishing Ebola cases from more common health conditions, such as malaria, have also reportedly complicated response efforts.⁴⁹ As of late September, over 80% of newly identified cases were reportedly among individuals who were not on existing contact-tracing lists, suggesting widespread ongoing transmission and significant gaps in existing surveillance efforts.⁵⁰ In mid-July, WHO warned that a similar statistic meant that the true scale of the outbreak could be “at least two to four times” as large as reported case data suggested.⁵¹

Complex security and political conditions in the main outbreak zone have posed significant challenges, despite DRC’s prior expertise with Ebola response. Dozens of armed groups are active in Ituri and North Kivu provinces (the heart of the outbreak zone), and conflicts have caused high levels of population displacement. Ituri also is a key gold mining area, spurring labor migration and cross-border travel (particularly to and from Uganda). The area has poor roads and other infrastructure gaps, which can delay response efforts.⁵²

As in the 2018-2020 Ebola outbreak in the same vicinity, community mistrust has fueled violent attacks on treatment centers, health workers, and burial teams.⁵³ Some health workers in the outbreak zone have reportedly gone unpaid and lack adequate protective equipment and other supplies, imposing new burdens on an overstretched workforce and prompting strikes and walk-outs.⁵⁴ These challenges are layered on top of entrenched public health struggles in DRC, including limited access to clean water and sanitation, significant other disease burdens (e.g., malaria and cholera), high rates of poverty and food insecurity, and shortfalls in state capacity.⁵⁵

Fractured territorial control has further impeded aid groups’ access and coordination. Most documented Ebola cases in the main outbreak zone to date have been in nominally government-held areas in Ituri and North Kivu. Armed groups exercise varying degrees of localized control in some of those areas, however, including a DRC-based affiliate of the Islamic State.⁵⁶ The M23 rebel group, backed by neighboring Rwanda, controls sizable territory in nearby areas of North and South Kivu, including the area’s two largest cities, Goma and Bukavu, and those cities’ airports, which might otherwise have served as logistical hubs for response activities. M23-held Goma also hosts one of DRC’s two high-level biomedical labs. The M23 has reportedly

⁴⁷ CDC, “Ebola Outbreak: Current Situation,” as of October 6, 2026.

⁴⁸ WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” September 10, 2026, and August 28, 2026, updates.

⁴⁹ The Global Fund, “Global Fund Approves Emergency Funding for Malaria Prevention to Safeguard Ebola Response in the Democratic Republic of the Congo,” July 16, 2026.

⁵⁰ Reuters, “Congo Ebola contact tracing falls far short as outbreak spreads, Africa CDC chief says,” September 25, 2026.

⁵¹ UN News, “‘This is a fire’: DRC Ebola outbreak is fastest-growing ever, warns WHO,” July 14, 2026.

⁵² *New York Times*, “‘No Help’: Aid Arrived Too Late as Ebola Spread in a Congolese Town,” October 5, 2026.

⁵³ See, for example, Associated Press (AP), “Safety fears limit Ebola response in Congo, with more than 12 attacks on health facilities and workers recorded,” July 18, 2026.

⁵⁴ Associated Press, “More health workers strike as Ebola cases in Congo near 3,000, including over 1,300 deaths,” July 25, 2026; Bloomberg, “Ebola Responders Reach Breaking Point in Hardest-Hit Congo Towns,” July 8, 2026.

⁵⁵ See, for example, World Bank, “Overview: DRC,” accessed July 27, 2026.

⁵⁶ See CRS In Focus IF10328, *The Islamic State and Its Affiliates*, by Clayton Thomas.

intimidated local health workers and inhibited information sharing and regional health authorities' access to areas it controls.⁵⁷

How has the United States responded to the outbreak at home?

U.S. Domestic Preparedness for Ebola Introduction

To date, no Ebola cases associated with the 2026 Bundibugyo virus outbreak have been reported in the United States.⁵⁸ The CDC and other U.S. Department of Health and Human Services (HHS) agencies have worked with partners to prepare for possible Ebola introduction into the United States, building upon ongoing public health emergency preparedness efforts.

In the United States, state, local, territorial, and tribal (SLTT) governments have the primary responsibility and authority to respond to emerging infectious disease threats in their respective jurisdictions. One main exception to this general rule is at borders and ports of entry, where HHS has primary authority for disease control.⁵⁹ As noted in the “Enhanced Airport Screenings for Certain Travelers” section below, since May, there has been enhanced airport screening in place to identify potentially infected or exposed travelers to the United States. The CDC staffs these airport screening sites and may act under regulations to protect against potential disease spread.⁶⁰ The CDC has also issued guidance to SLTT public health departments for assessment and ongoing monitoring after airport screening of individuals who have traveled to areas near the Ebola outbreak.⁶¹

A possible U.S. Ebola case may first be identified by a health care provider within the community. The early signs and symptoms of Ebola may be mistaken for other diseases, which may delay health care providers in identifying potential cases and possibly contribute to disease spread. The CDC has published guidance for health care providers on identifying and investigating potential cases and has used its Health Alert Network system to disseminate this guidance throughout the country.⁶² Once a suspected Ebola case is identified, U.S. health care providers are to report the case to their respective SLTT public health department, which may then potentially facilitate confirmatory testing for Ebola, further assessment, and further public health actions as needed (e.g., patient isolation).⁶³ Viral hemorrhagic fever caused by Ebola virus is a nationally notifiable condition, which means that all states are recommended to have laws in place to require reporting of potential Ebola cases.⁶⁴ SLTT public health departments are advised to coordinate with the CDC's Viral Special Pathogens Branch when an Ebola case is suspected to ensure appropriate investigations and precautions to prevent disease spread.⁶⁵

⁵⁷ *New York Times*, “How Rebels in Congo Could Use Ebola to Consolidate Their Power,” July 8, 2026.

⁵⁸ CDC, “Ebola Outbreak: Current Situation,” as of October 6, 2026.

⁵⁹ CDC, “Legal Authorities for Isolation and Quarantine,” May 15, 2024, and Department of Homeland Security, *Biological Incident Annex to the Response and Recovery Interagency Operational Plan*, May 2023, pp. 7-8.

⁶⁰ 42 C.F.R. Part 71.

⁶¹ CDC, “Interim Guidance for Public Health Assessment and Management of Travelers from Countries Affected by the 2026 Ebola Outbreak,” August 7, 2026, <https://www.cdc.gov/ebola/php/emergency-guidance/index.html>.

⁶² CDC, “Ebola Disease Outbreak in the Democratic Republic of the Congo and Uganda,” *Health Alert Network*, May 19, 2026.

⁶³ CDC, “Public Health Management of People with Suspected or Confirmed VHF or High-risk Exposures,” August 7, 2026.

⁶⁴ CDC, “2026 National Notifiable Conditions.”

⁶⁵ CDC, “Public Health Management of People with Suspected or Confirmed VHF or High-risk Exposures.”

The CDC has supported U.S. SLTT public health capacity to identify and contain Ebola through several grant programs. For example, the CDC’s Epidemiology and Laboratory Capacity cooperative agreement program and its Public Health Emergency Preparedness cooperative agreement program help fund the network of public health laboratories across the country with capacity to test for Ebola viruses, along with staff at the state, territorial, and local public health agencies with expertise to investigate and respond to potential Ebola cases.⁶⁶

A different HHS agency, the Administration for Strategic Preparedness and Response (ASPR) has funded several grant programs that support health care system capacity to contain disease spread from Ebola patients.⁶⁷ Most notably, ASPR has funded the National Special Pathogen System (NSPS), which supports a tiered system of health care facilities for managing patients with dangerous infectious diseases such as Ebola.⁶⁸ Within this system, 13 Regional Emerging Special Pathogen Treatment Centers around the country can provide highly specialized care with advanced infection isolation capabilities to prevent Ebola spread from patients. There are plans and protocols in place to transfer Ebola patients from anywhere in the country to these specialized facilities as needed.⁶⁹ ASPR first established parts of NSPS in 2015, during the 2014-2016 West Africa Ebola outbreak, and since then NSPS has evolved into a coordinated national system.⁷⁰ As of September 1, 2026, ASPR awarded additional funds to expand the number of health facilities with infection control capabilities in NSPS.⁷¹

HHS agencies are also involved in developing potential vaccines and treatments for Ebola disease caused by Bundibugyo virus, as discussed in “What is the current status of vaccines and therapeutics? How is the United States participating in such efforts?” below.

U.S. Travel Restrictions

The executive branch has taken various actions to restrict travel to the United States from areas affected by the Ebola outbreak. The stated purpose of these restrictions is to “protect the health of the United States from the serious risk posed by the introduction of Ebola disease into the United States.”⁷² The travel restrictions generally prohibit individuals who have been in specified countries within the past 21 days from boarding a flight to or entering the United States. These individuals generally must spend at least 21 days outside the specified countries and the United States before they can board a flight to or enter the United States. For foreign nationals, the specified countries are DRC, Uganda, and South Sudan. For U.S. citizens, the specified country is limited to DRC. U.S. citizens who have been present in Uganda and South Sudan within the past 21 days are permitted to board flights to the United States but are subject to enhanced airport screening upon arrival (see “Enhanced Airport Screenings for Certain Travelers”).

⁶⁶ CDC, “The Epidemiology and Laboratory Capacity (ELC) Program,” <https://www.cdc.gov/epidemiology-laboratory-capacity/php/about/index.html>; and CDC State and Local Readiness, “2024-2028 PHEP Cooperative Agreement Guidance/Budget Period 1.”

⁶⁷ For a summary of ASPR’s health care readiness programs, see the Health Care Readiness and Recovery section of ASPR’s Congressional Justification FY2027, pp. 39-45.

⁶⁸ National Emerging Special Pathogens Training and Education Center (NETEC), “National Special Pathogen System,” <https://netec.org/nsps/>.

⁶⁹ NETEC, “About the NSPS,” and Administration for Strategic Preparedness and Response, “ASPR Supports Global Ebola Response While Strengthening U.S. Preparedness,” June 26, 2026.

⁷⁰ NSPS, “About the NSPS,” and NETEC, “National Special Pathogen System (NSPS) Strategy,” February 1, 2024.

⁷¹ ASPR, “ASPR and NETEC Announce Second Round of NSPS Level 2 STAND Awardees to Strengthen Special Pathogen Preparedness Nationwide,” September 1, 2026.

⁷² CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Suspending Introduction of Certain Persons From Countries Where a Communicable Disease Exists,” 99 *Federal Register* 29961, May 21, 2026.

As during previous Ebola outbreaks, the WHO has criticized travel bans as having “no basis in science.”⁷³ WHO has also argued that travel bans result in people and goods turning to informal border crossings that are not monitored and increase the chances that the disease will spread. Some public health experts have warned that restrictions can disrupt the movement of health care workers and supplies, thus potentially inhibiting international response efforts.⁷⁴

U.S. Travel Restrictions for Foreign Nationals

On May 18, 2026 (the same day that the WHO Director General declared the current outbreak a Public Health Emergency of International Concern), the CDC issued an order prohibiting the entry of certain aliens into the United States for 30 days.⁷⁵ It stated that the order was necessary “to protect the health of the United States from the serious risk posed by the introduction of Ebola disease into the United States ... based on the emergent outbreak of Ebola disease caused by the Bundibugyo virus strain confirmed present in Democratic Republic of the Congo (DRC) and Uganda.”⁷⁶ The order was issued pursuant to communicable disease control provisions in the Public Health Service Act.⁷⁷

The order applied to “covered aliens” who had been present in the DRC, Uganda, or South Sudan during the past 21 days, regardless of their country of origin. The order did not apply to U.S. citizens, U.S. nationals,⁷⁸ lawful permanent residents (LPRs, also known as *green card holders*), members of the U.S. armed forces and their immediate family members, or U.S. government personnel serving overseas and their immediate family members. The order provided for certain exceptions, including for “law enforcement, officer and public safety, humanitarian, and public health interests,” as well as those who “are permitted to enter the United States as part of a Department of Homeland Security (DHS)-approved process.” The order instructed DHS—which has the primary responsibility for implementing and managing operations at ports of entry—to share information and consult with the CDC about exceptions and associated mitigation protocols.

On May 22, the order was amended to include lawful permanent residents among those prohibited from entering the United States.⁷⁹ The amended order stated that the reason for expanding the prohibition was to reduce the number of travelers who would need to be screened and monitored and to minimize the risk of introducing Ebola to the United States. This amended

⁷³ WHO, “Epidemic of Ebola Disease caused by Bundibugyo virus in the Democratic Republic of the Congo and Uganda determined a public health emergency of international concern,” May 17, 2026.

⁷⁴ ABC News, “Why travel restrictions may have unintended consequences as US bars some visitors amid Ebola outbreak,” May 23, 2026.

⁷⁵ *Alien* is the term used in the Immigration and Nationality Act (INA) to mean anyone who is not a citizen or national of the United States. 8 U.S.C. § 1101(a)(3). It is synonymous with foreign national and includes lawful permanent residents (LPRs).

⁷⁶ CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Suspending Introduction of Certain Persons From Countries Where a Communicable Disease Exists,” 99 *Federal Register* 29961, May 21, 2026.

⁷⁷ The order specifically cited Sections 362 and 365 of the Public Health Service Act, 42 U.S.C. §§ 265, 268, and their implementing regulations under 42 C.F.R. part 71.

⁷⁸ U.S. nationals are persons owing permanent allegiance to the United States (including U.S. citizens). Noncitizen nationals are individuals who were born either in American Samoa or on Swains Island to parents who are not citizens of the United States.

⁷⁹ CDC, “Notice of an Amended Order Under Sections 362 and 365 of the Public Health Service Act Suspending Introduction of Certain Persons From Countries Where a Communicable Disease Exists,” 99 *Federal Register* 29961, May 29, 2026.

order was in effect for 30 days. Since that time, the CDC has issued orders monthly to renew these travel restrictions for 30 days at a time.⁸⁰

Enhanced Airport Screenings for Certain Travelers

In May 2026, DHS directed all flights carrying individuals (regardless of nationality) who had been present in DRC, Uganda, or South Sudan within the past 21 days to land at Washington-Dulles International Airport (IAD).⁸¹ (There are no direct, nonstop commercial flights to the United States from these countries; the directive thus applies to individuals entering via a connecting flight.) The announcement stated that IAD receives the highest number of travelers from these countries and that DHS and HHS had implemented enhanced public health measures there. DHS later modified the list of airports to include Hartsfield-Jackson Atlanta International Airport (ATL), George Bush Intercontinental Airport (IAH) in Houston, and John F. Kennedy International Airport (JFK) in New York City.⁸² Subsequently, on August 31, CBP removed IAH from the list of designated airports.⁸³

Once these travelers arrive in the United States, they are subject to a health screening and questionnaire process with potential additional assessments depending on their area of travel and activities within the Ebola-affected country.⁸⁴ As further explained below, as of July, this enhanced airport screening applies only to travelers who have been present in Uganda or South Sudan within 21 days of arrival in the United States; individuals who have been in DRC within 21 days are subject to further restrictions.⁸⁵

U.S. Travel Restrictions for All Travelers from the DRC, Including U.S. Citizens

In July, the CDC announced that it would also begin barring U.S. citizens and nationals who had been present in DRC in the past 21 days from boarding U.S.-bound commercial flights.⁸⁶ This policy applies to all travelers who have been in DRC, including U.S. citizens.⁸⁷ Such persons must spend 21 days in a third country before being allowed to enter the United States (for Americans known to have been exposed to Ebola in DRC, this might include 21 days in the

⁸⁰ CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Continuing the Suspension of the Right To Introduce Certain Persons From Countries Where a Quarantinable Communicable Disease Exists,” 91 *Federal Register* 38441, June 25, 2026; CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Continuing the Suspension of the Right To Introduce Certain Persons From Countries Where a Quarantinable Communicable Disease Exists,” 91 *Federal Register* 43636, July 16, 2026; CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Continuing the Suspension of the Right To Introduce Certain Persons From Countries Where a Quarantinable Communicable Disease Exists,” 91 *Federal Register* 53252, August 17, 2026; and CDC, “Order Under Sections 362 and 365 of the Public Health Service Act Continuing the Suspension of the Right To Introduce Certain Persons From Countries Where a Quarantinable Communicable Disease Exists,” 91 *Federal Register* 58673-58679, September 16, 2026.

⁸¹ DHS, U.S. Customs and Border Protection (CBP), “Arrival Restrictions Applicable to Flights Carrying Persons Who Have Recently Traveled From or Were Otherwise Present Within the Democratic Republic of the Congo (DRC), Uganda, or South Sudan,” 91 *Federal Register* 29896-29897, May 21, 2026.

⁸² CBP, “Modification of List of Designated Airports Regarding Arrival Restrictions Published May 24, 2026.”

⁸³ UCBP, “Modification of List of Designated Airports Regarding Arrival Restrictions Published August 31, 2026.” See <https://www.cbp.gov/newsroom> for updates.

⁸⁴ CDC, “Information for Travelers Returning from Ebola-Affected Areas,” <https://www.cdc.gov/ebola/situation-summary/returning-travelers.html>.

⁸⁵ CDC, “Information for Travelers Returning from Ebola-Affected Areas.”

⁸⁶ CDC, X Post, July 16, 2026, <https://x.com/CDCgov/status/2077471983940313266?s=20>; CDC, “Information for Travelers Returning from Ebola-Affected Areas.”

⁸⁷ CDC, “Information for Travelers Returning from Ebola-Affected Areas.”

Kenya quarantine facility, explained in greater detail in “Kenya Quarantine Facility”). This restriction applies to all passengers with a stop in DRC, regardless of whether they disembarked the airplane.⁸⁸ This restriction is implemented through the Do Not Board (DNB) list, established by the CDC in cooperation with DHS in 2007 to prevent individuals with a communicable disease that poses a serious threat to the public from boarding commercial aircraft departing from or arriving to the United States.⁸⁹ Individuals placed on the Do Not Board list are also issued a Public Health Lookout, which alerts CBP to notify public health officials when an individual subject to the Lookout attempts to enter the United States at a port of entry, including land borders.⁹⁰ Federal quarantine law authorizes the CDC, under specified conditions to prevent the spread of certain communicable diseases, to detain, medically examine, and conditionally release individuals arriving into the United States.⁹¹ According to media reporting, the CDC has temporarily detained certain U.S. citizens traveling back to the United States through land borders who had been present in the DRC within 21 days of attempting to reenter the United States.⁹²

How has the United States responded to the outbreak overseas?

U.S. Foreign Assistance

The State Department reports that the United States is “the largest provider of health and humanitarian assistance” to the current Ebola response.⁹³ As of September 23, 2026, the State Department has announced more than \$887 million for the response in Africa, of which more than \$780 million is for “direct health and humanitarian assistance.”⁹⁴ The most recent tranche of U.S. funding—\$267 million—was announced in September 2026 at the 81st session of the United Nations General Assembly High-level week alongside other donor pledges (for more on other donors, see “How, if at all, are other aid donors responding to the crisis?”).⁹⁵ According to the State Department, such funds have supported implementing partners in

- disease surveillance and detection, including contact tracing, border screenings, and increased diagnostic capacity at laboratories;

⁸⁸ Department of State, “Additional Consular Information for Americans Regarding the Ebola Outbreak,” updated August 22, 2026.

⁸⁹ CDC, “Travel Restrictions to Prevent the Spread of Contagious Diseases,” last updated May 15, 2024, <https://www.cdc.gov/port-health/travel-restrictions/index.html>. According to CDC’s website, “the Transportation Security Administration (TSA) enforces [the Do Not Board] list. Under the Aviation and Transportation Security Act (49 U.S.C. § 114), TSA may take actions necessary to reduce the risk of threats to aviation and transportation security. This includes denying boarding to travelers CDC identifies as having or likely having a contagious disease that poses a public health threat to other passengers or crew.”

⁹⁰ CDC, “Travel Restrictions to Prevent the Spread of Contagious Diseases,” last updated May 15, 2024, <https://www.cdc.gov/port-health/travel-restrictions/index.html>.

⁹¹ Public Health Service Act Section 361(b) (42 U.S.C. §264); 42 C.F.R. §§71.36–71.39.

⁹² See, for example, Katherine Eban, “Trump’s CDC Is Detaining Americans in Response to Ebola: A ‘Wild Misuse’ of Power,” *Rolling Stone*, July 21, 2026.

⁹³ Department of State, “United States Pledges Additional Life-Saving Health Assistance in Response to Ebola Outbreak,” September 23, 2026. Implementing partners include international organizations such as UNICEF and the International Organization for Migration, as well as nongovernmental organizations such as MedAir, FHI 360, and International Medical Corps. Department of State, “Ebola Response Update – June 10, 2026.”

⁹⁴ Department of State, “United States Pledges Additional Life-Saving Health Assistance in Response to Ebola Outbreak,” September 23, 2026.

⁹⁵ Kerry Cullinan, “Almost \$3 billion Raised For DRC’s Ebola Outbreak, as Focus Shifts to Nord-Kivu,” *Health Policy Watch*, September 24, 2026.

- the establishment and operation of treatment clinics and other health facilities in Ebola-affected areas, including the procurement and delivery of protective equipment, diagnostics, and other “critical health commodities”;
- the establishment and deployment of “safe and dignified burial teams”; and
- “risk communication messaging,” in large part through community and religious leaders to raise awareness of the disease, its spread, and potential treatment.⁹⁶

In June 2026, State committed \$50 million to the Coalition for Epidemic Preparedness Innovations (CEPI) for Bundibugyo vaccine development efforts, including funding for laboratory studies and clinical trials.⁹⁷ The State Department reported in May that it was also “actively collaborating with American private sector companies and international actors to rapidly scale up diagnostic testing capacities on the ground,” and working with U.S. private sector firms “to support efforts to test and deploy the most promising existing anti-viral treatments” for the Bundibugyo virus causing this Ebola outbreak.⁹⁸

The Department also has announced \$350 million for “critical humanitarian assistance in the DRC, South Sudan, and Uganda.”⁹⁹ This draws from the \$1.8 billion in funding the Department announced for the UN Office for the Coordination of Humanitarian Affairs in May 2026.¹⁰⁰

The CDC is also working abroad to respond to the Ebola outbreak. It has accessed \$107 million from the Infectious Disease Rapid Response Reserve Fund to support the response.¹⁰¹ More than 120 CDC staff are deployed to the region and are providing “strategic and technical assistance” for various efforts, including disease tracking and contact tracing, laboratory sample collection and virus sequencing, infection prevention and control, and “risk communication and community engagement.”¹⁰² The CDC notes that it continues to do its international work in coordination with “U.S. government agencies, health ministries in affected countries, humanitarian organizations, and other partners.”¹⁰³ The extent to which U.S. personnel are able to travel to the outbreak zone is unclear.

Outside of the State Department’s contribution to CEPI, the U.S. government has supported vaccine and therapeutic development. This is discussed in greater detail in “What is the current status of vaccines and therapeutics? How is the United States participating in such efforts?”

⁹⁶ Department of State, “United States Pledges Additional Life-Saving Health Assistance in Response to Ebola Outbreak,” September 23, 2026; Department of State, “Ebola Response Update – August 5, 2026”; Department of State, “State Department Ebola Response Updates,” accessed on August 25, 2026.

⁹⁷ Department of State, “Ebola Response Update – June 12, 2026,” and CEPI, “U.S. Department of State commits \$50 million to CEPI to fast-track Bundibugyo virus medical countermeasures,” June 11, 2026.

⁹⁸ Department of State, “Ebola Response Update – May 23, 2026.”

⁹⁹ Department of State, “Ebola Response Update – August 5, 2026.”

¹⁰⁰ Department of State, “United States Pledges Additional \$1.8 Billion in Life-Saving Humanitarian Funding to OCHA’s Humanitarian Reset & Shares Powerful Results from the Trump Administration’s UN Humanitarian Reform Agreement,” May 14, 2026.

¹⁰¹ CDC, “Transcript - Update on Ebola Outbreak in the Democratic Republic of the Congo and Uganda and CDC’s World Cup 2026™ activation,” press release, June 18, 2026. For more on the Fund, see the relevant section in CRS Report R47207, *Centers for Disease Control and Prevention (CDC) Funding Overview*, by Kavya Sekar and Alexa C. DeBoth.

¹⁰² CDC, “Ebola Outbreak: What CDC is Doing,” accessed on September 1, 2026.

¹⁰³ CDC, “Ebola Outbreak: Current Situation,” updated on September 25, 2026; and CDC, “Ebola Outbreak: What CDC is Doing,” accessed on September 1, 2026.

Kenya Quarantine Facility

Related to “U.S. Travel Restrictions” above, the Trump Administration has constructed a facility in Kenya to quarantine and potentially treat U.S. citizens exposed to Ebola, with the stated goal that it is “to prevent the Ebola outbreak from reaching our shores.”¹⁰⁴ The plan has prompted protests and legal challenges in Kenya, whose High Court issued an order in late May temporarily barring the facility’s establishment pending further court action.¹⁰⁵ Kenyan officials have defended the project, describing the unit, located at Laikipia Air Base in central Kenya, as a military facility that would serve both U.S. and Kenyan nationals.¹⁰⁶

The U.S. Embassy in Kenya issued its first statement about the Ebola facility on June 2, declaring that U.S. officials were “actively working with the Kenyan government to resolve any objections and communicate our shared objectives to the Kenyan people.”¹⁰⁷ The embassy statement described the facility as part of a “holistic response to prevent the spread of the disease and lessen health risks for the region as a whole,” and it stated that the facility would not pose a risk to nearby communities. In late June, Kenya’s High Court reportedly found the country’s health minister in contempt of court for failing to halt construction of the facility.¹⁰⁸

In July, seven American aid workers who had worked on the Ebola response in DRC with the U.S.-based charity Samaritan’s Purse were quarantined for weeks in the U.S. facility in Kenya—it was the first known use of the facility and a potential violation of the Kenyan court order.¹⁰⁹ None developed Ebola and all were eventually discharged.

The Trump Administration has not publicly released details on the cost of the facility or how it was funded, though reports suggest the facility was built for approximately \$70 million.¹¹⁰ DOD appears to have played a role in its construction and funding.¹¹¹ Separately, the United States has to date pledged \$14.5 million in foreign assistance specifically for Kenya’s Ebola preparedness efforts.¹¹² The Administration also requested supplemental funding for the Kenya facility in its June request (see “How is the United States funding its international response?”).

How is the United States funding its international response?

Funding for the State Department’s response to date appears to be drawn from regular annual appropriations for the Global Health Programs (GHP), International Humanitarian Assistance (IHA, for FY2026), and International Disaster Assistance (IDA, for FY2025 and prior years) accounts. Some emergency funding appropriated in FY2015 in response to the Ebola outbreak in

¹⁰⁴ Department of State, “Secretary Rubio’s Call with Kenyan President Ruto,” May 28, 2026. See also Secretary of State Marco Rubio’s testimony before the Senate Appropriations Subcommittee on State, Foreign Operations, and Related Programs, FY2027 State Department Budget Request, hearing, June 3, 2026.

¹⁰⁵ Juristnews, “Kenya dispatch: High Court suspends US-backed Ebola quarantine facility,” June 6, 2026.

¹⁰⁶ Citizen (Nairobi), “Duale defends Laikipia Ebola facility, says it will serve both Kenyans and Americans,” May 30, 2026.

¹⁰⁷ U.S. Embassy in Kenya, “Statement on Ebola Response Efforts and Cooperation with Kenya,” June 3, 2026.

¹⁰⁸ Reuters, “US equipment, experts arrive at Kenya Ebola facility despite court order, protests,” June 3, 2026.

¹⁰⁹ *Politico*, “Americans who responded to Ebola outbreak are quarantined in Kenya, despite court order,” July 17, 2026.

¹¹⁰ Yasmeen Abutaleb et al., “How Trump’s America First policy hit the Ebola response in Congo,” Reuters, October 6, 2026.

¹¹¹ Reuters, “US equipment, experts arrive at Kenya Ebola facility despite court order, protests,” June 3, 2026.

¹¹² Department of State, “Secretary Rubio’s Call with Kenyan President Ruto,” May 28, 2026, and “Ebola Response Update,” June 12, 2026. U.S. Embassy in Kenya, “U.S. Support for Kenya’s Ebola Virus Disease Preparedness,” July 10, 2026.

West Africa has reportedly also been used.¹¹³ As noted above, the CDC international response appears to be funded primarily through the Infectious Disease Rapid Response Reserve Fund (IDRRRF), which may include multiyear and no-year funding authority.¹¹⁴ Additional funding might also be drawn from other regular annual CDC appropriations, such as the Emerging and Zoonotic Infectious Diseases and Global Health appropriations. ASPR and BARDA support of vaccines and therapeutic development also appear to be drawn from regular appropriations, particularly for Research, Development and Procurement, but may also be funded by transfers from the IDRRRF.

On June 24, 2026, the Trump Administration transmitted to Congress a request for \$87.6 billion in supplemental FY2026 funding, including \$1.44 billion to address the Ebola outbreak in Central Africa.¹¹⁵ Of that amount, \$800 million was requested in IHA funding to respond to the outbreak and provide for the Ebola facility in Kenya; \$550 million in GHP funding to “support contact tracing, personal protective equipment and commodity procurement, disease surveillance, laboratory capacity, and cross-border coordination”; and \$90 million in Emergencies in the Diplomatic and Consular Service funding for “unanticipated needs related to the Ebola situation including for the medical evacuation of U.S. citizens.”¹¹⁶ The Administration did not request additional funding for the CDC response.

As of this writing, Congress has not introduced legislation that would provide the requested funding.

How is the outbreak affecting other U.S. foreign policy priorities in Central Africa?

Responding to the current Ebola outbreak has drawn U.S. resources and senior policymakers’ attention at a time of competing U.S. policy priorities in DRC and neighboring countries. The outbreak, and some elements of the U.S. response, may distract from or inhibit the pursuit of other U.S. goals. These include the Trump Administration’s efforts to stabilize eastern DRC, promote U.S. commercial investment there and elsewhere in Africa, and curtail U.S. foreign assistance spending.¹¹⁷ How the Ebola outbreak might affect activities under the U.S.-DRC bilateral health cooperation memorandum of understanding (MOU), signed in February 2026, is also uncertain. The MOU identifies “disease surveillance and outbreak response” as one of several priorities for U.S. health aid in the coming years.¹¹⁸ The status of implementation and funding obligations to date under the MOU have not been made public.

¹¹³ During the 2014-2016 Ebola outbreak in West Africa, the United States mobilized a whole-of-government response. Congress enacted more than \$5.4 billion in FY2015 emergency supplemental appropriations to support the response. Because Congress provided such funding with the emergency designation, it was not subject to congressionally established discretionary budget caps. Further, because some of the funds were made available until expended (i.e., did not have a date by which they were required to be obligated), some funds have remained unused.

¹¹⁴ For more in the Infectious Disease Rapid Response Reserve Fund, see the relevant section in CRS Report R47207, *Centers for Disease Control and Prevention (CDC) Funding Overview*, by Kavya Sekar and Alexa C. DeBoth.

¹¹⁵ June 24, 2026, letter from OMB Director Russell Vought to Speaker Mike Johnson, <https://www.whitehouse.gov/wp-content/uploads/2026/06/2026.06.24-Letter-to-the-Honorable-Mike-Johnson.pdf>.

¹¹⁶ June 24, 2026, letter from OMB Director Russell Vought to Speaker Mike Johnson, p. 3 of “Urgent Supplemental Funding Needs.”

¹¹⁷ See, for example, remarks by Nick Checker, then-Senior Bureau Official for African Affairs, “A New Model for Economic Prosperity in Africa,” Department of State, April 10, 2026.

¹¹⁸ Under the MOU, the State Department “intends to provide” up to \$900 million in bilateral health aid over the next five years, while the DRC government committed to increase its own domestic health funding by \$300 million. (continued...)

The Administration has devoted high-level efforts to ending the conflict between DRC and the Rwanda-backed M23 rebel group, and brokered a peace deal between DRC and Rwanda in 2025.¹¹⁹ The M23 controls territory in eastern DRC and has reported cases within areas it holds; the group has reportedly relied on Rwanda for supplies and technical support.¹²⁰ Efforts to engage the DRC government, regional governments, and (potentially) M23 leaders on Ebola response may distract from other U.S. objectives and could pose policy quandaries, such as whether to work with the M23 on Ebola containment efforts at the risk of bolstering a nonstate armed group whose insurgency the United States has sought to curtail. The U.S. Ebola-related travel restrictions have also led the Administration to relocate recent DRC-Rwanda peace accord implementation talks from Washington, DC, to Europe.¹²¹

Bolstering U.S. investment in DRC's mining sector has been another top goal of the Trump Administration, in part as a means to counter the People's Republic of China and its influence over DRC's mineral supply chains.¹²² DRC is the world's top mine producer of cobalt and tantalum and the second largest of copper, and the country also has reserves of lithium and other U.S.-designated "critical minerals."¹²³ While DRC's industrial copper-cobalt mining area is located about 1,000 miles to the south of the Ebola outbreak, other minerals (such as tantalum) are mined in parts of eastern DRC that have seen Ebola cases, and the primary outbreak zone in Ituri is a key gold-mining region. The Ebola outbreak may increase DRC's risk profile for prospective U.S. investors, while regional border closures linked to Ebola may complicate industrial activities and trade. The U.S. travel restrictions may disrupt other trade and investment facilitation, such as commercial delegations.

DRC is also one of several African countries that have entered into agreements with the Trump Administration to accept third-country nationals removed from the United States.¹²⁴ In April 2026, the U.S. government reportedly transferred 15 individuals to DRC under the agreement.¹²⁵ In May, the Administration reportedly suspended such transfers to DRC due to concerns about the risk of Ebola exposure for U.S. Immigration and Customs Enforcement personnel accompanying

Department of State, "Fostering Health Sovereignty in the Democratic Republic of the Congo Through the America First Global Health Strategy," February 26, 2026, and "America First Global Health Strategy," September 2025.

¹¹⁹ See Department of State, "Signing of the Washington Accords for Peace and Prosperity Between the Democratic Republic of the Congo and Rwanda," December 4, 2025. For additional background, see CRS Insight IN12495, *Conflict Surge in Eastern Democratic Republic of Congo: Issues for Congress*, and CRS Insight IN12619, *Peace Process in Eastern Democratic Republic of Congo: Issues for Congress*.

¹²⁰ Reuters, "Rebel-held eastern Congo records new Ebola case after declaring outbreak over," September 4, 2026; and Reuters, "Rebels in Congo see Ebola as chance to show governing credentials," July 13, 2026.

¹²¹ Department of State, "Joint Statement on the Sixth Meeting of the Joint Security Coordination Mechanism for the Peace Agreement Between the Democratic Republic of the Congo and the Republic of Rwanda," August 13, 2026.

¹²² Department of State, "Strategic Partnership Agreement Between the Government of the United States of America and the Government of the Democratic Republic of the Congo," "Regional Economic Integration Framework Between the Government of the Democratic Republic of the Congo and the Government of the Republic of Rwanda," and "Joint Statement Declaration of Intent from the United States Government and the European Commission on the Lobito Corridor and Investment in the Great Lakes Region," December 4, 2025; see also, for example, Reuters, "Congo, China deepen mining ties as US pushes rival minerals pact," March 27, 2026.

¹²³ U.S. Geological Survey (USGS), "Mineral Commodity Summaries 2026," May 2026.

¹²⁴ See CRS Report R49319, *China's Diplomacy: Selected Issues for Congress*, by Ricardo Barrios. An exchange of diplomatic notes outlining the agreement is on the State Department's website at <https://foia.state.gov/FOIALIBRARY/QN12.aspx> (see "2026-0029QN - Congo DROC..." within the "April 2026 (zip)").

¹²⁵ *New York Times*, "U.S. Migrants Deported to Congo: 'Where on Earth Is This Place?'" May 15, 2026.

such transfers, as well as the potential that removed individuals could bring Ebola to their home countries in South America (closer to the United States) if DRC ultimately repatriates them.¹²⁶

How has the World Health Organization (WHO) responded to the outbreak? Has the United States engaged with WHO?

WHO is playing a key role in coordinating the global response to the Ebola crisis in the DRC. On May 17, 2026 (two days after the DRC government declared and confirmed the Bundibugyo outbreak), WHO Director-General (DG) Tedros Adhanom Ghebreyesus designated the outbreak as a Public Health Emergency of International Concern (PHEIC), which requires international coordination and cooperation under the International Health Regulations.¹²⁷ WHO is coordinating with the DRC national government in Kinshasa as well as at the provincial levels, including with local health authorities; it is also engaging with global, regional, and local partners outside government to provide emergency coordination, community protection, collaborative disease surveillance, and treatment services. This includes working with community and traditional leaders to raise disease awareness, providing medical supplies and contact tracing, establishing isolation and treatment centers near impacted communities, and supporting safe burials.¹²⁸ In June 2026, WHO launched a joint continental preparedness and response plan with the Africa Centres for Disease Control and Prevention (Africa CDC), which broadly aims to support African countries in preparing for, detecting, and responding to infectious disease outbreaks, including the ongoing Ebola outbreak.¹²⁹ The full extent of WHO funding for the outbreak is unclear. A key funding stream includes the WHO Contingency Response for Emergencies (CFE), which released \$3.9 million for the DRC crisis as of June 30, 2026.¹³⁰

The United States withdrew from WHO in January 2026 and does not provide direct funding to or participate in the organization.¹³¹ During the current DRC outbreak, the United States has

¹²⁶ *Politico*, “Trump administration temporarily pauses deportation flights to Congo amid Ebola outbreak,” May 21, 2026.

¹²⁷ The International Health Regulations (IHR) are an overarching legal framework that defines the rights and obligations of States Parties (including the United States) in handling public health events and emergencies that have the potential to cross borders. The IHR was adopted and is periodically amended by the World Health Assembly (the WHO plenary body). Among other things, IHR outlines criteria for declaring a public health emergency of international concern (PHEIC), requiring States Parties to report public health events, designate focal points for communication with WHO, and maintain core capacities for surveillance and response. The United States remains a State Party to IHR (2005); however, it has rejected amendments on pandemic response adopted in 2024. For more information, see CRS In Focus IF12139, *International Health Regulations Amendments*, by Tiaji Salaam-Blyther and Matthew C. Weed, and CDC, “International Health Regulations,” at <https://www.cdc.gov/global-health/topics-programs/ihr.html>.

¹²⁸ Specifically, WHO has worked with partners to deploy more than 260 experts into the field; deliver more than 300 metric tons of medical and operational supplies; establish isolation and treatment centers close to the affected communities; and support vaccine research and development, including coordinating a clinical trial in Ituri province in three different facilities to find effective treatments. See WHO, “Bundibugyo virus disease outbreak - Democratic Republic of the Congo,” <https://www.afro.who.int/health-topics/ebola-disease/outbreak-drc-26>.

¹²⁹ The six-month plan (from June to November 2026) aims to raise \$518 million to support African countries (and partners) in Ebola preparedness and response. See WHO and Africa CDC, *Bundibugyo Ebola virus, Continental Preparedness and Response Plan: June-November 2026*, June 2026.

¹³⁰ WHO, “Contingency Fund for Emergencies (CFE) Mid-Year Update,” July 2026.

¹³¹ The United States has historically been a WHO member and the organization’s largest financial contributor. On January 20, 2025, President Trump issued Executive Order 14155 stating that the United States would withdraw from WHO, citing what he characterized as the “mishandling” of the COVID-19 pandemic, failure to adopt reforms, and unfair funding practices. The President paused the future transfer of any U.S. funding, support, or resources to WHO, and recalled and reassigned U.S. personnel and contractors working at WHO. Under the U.S. withdrawal process outlined by a joint resolution of Congress (22 U.S.C. §290c), the United States announced its formal withdrawal from (continued...)

coordinated and shared relevant information with WHO and other organizations as part of broader cooperation with partners in the field. It has also been in contact with WHO regarding the status of U.S. citizens that tested positive for the virus.¹³² These activities appear to be in line with U.S. obligations and activities under the 2005 International Health Regulations and similar to recent U.S.-WHO engagement on other global health situations such as the May 2026 multicountry Hantavirus cluster.¹³³ During previous Ebola outbreaks, the United States provided direct communication, funding, and technical support to WHO.¹³⁴ Some observers and U.S. policymakers have expressed concern that comparatively limited U.S.-WHO engagement during the current outbreak might slow the pace and effectiveness of the Ebola response.¹³⁵ At this time, the possible effects, if any, are unclear.¹³⁶

How, if at all, are other aid donors responding to the crisis?

While the United States is the largest donor to the current Ebola response, a range of governments, international organizations, and other entities have pledged or provided funding to respond to the crisis in the DRC. WHO and Africa CDC initially estimated in June 2026 that \$518 million was required to contain the outbreak; however, officials subsequently increased the estimate to \$1.4 billion to account for growing health and humanitarian needs.¹³⁷ As of September 24, 2026, Africa CDC reported that commitments from governments and other donors reached \$2.997 billion.¹³⁸ Examples of pledges include

- \$568 million from the European Union/European Commission, including for emergency humanitarian assistance;
- over \$433 million from the World Bank (for DRC and the region, in collaboration with WHO and Africa CDC); and
- contributions from other entities such as the Pandemic Fund (\$221 million), CEPI (\$196 million), and UN Central Emergency Response Fund (\$90 million).¹³⁹

Governments have also committed funding to help contain the outbreak, including Germany (\$127 million), several African countries (\$110 million), the United Kingdom (\$105 million), and

WHO on January 22, 2026. U.S. Department of Health and Human Services, “Fact Sheet: U.S. Withdrawal from the World Health Organization,” last reviewed on January 22, 2026.

¹³² WHO, “WHO Director-General’s address to Member States at the 79th World Health Assembly,” May 19, 2026.

¹³³ WHO official Abdi Mahamud, director of WHO’s health emergency alert and response operations reportedly stated on the hantavirus: “In terms of collaboration with U.S. and U.S. institutions, it has been going very well.... The information flow is there, transparent and frank information sharing.” Drawn from WHO, “WHO Director-General’s remarks at the media briefing,” May 7, 2026, and WHO Media Briefing transcript at <https://www.youtube.com/live/ksOTrl0zM5A?si=ySU0wA49iVohitK->.

¹³⁴ For example, see pages 7 and 43 of GAO report GAO-18-350, *Ebola Recovery: USAID Has Initiated or Completed Most Projects, but a Complete Project Inventory Is Still Needed for Evaluating Its Efforts*, March 2018.

¹³⁵ For example, see NPR, “U.S. aid cuts may have delayed detecting this Ebola outbreak,” May 21, 2026, and Joseph Choi, “Ebola response hobbled by US withdrawal from global health,” *The Hill*, May 24, 2026.

¹³⁶ See, for example, H.R. 10345 [119th], EBOLA Act, September 10, 2026 (S. 4929 in the Senate), which calls on the President to rejoin the World Health Organization and collaborate with WHO.

¹³⁷ Anait Miridzhanian and Sfundu Parakozov, “Africa CDC Says Funding Needs for Ebola response three times higher at \$1.4 billion,” Reuters, June 25, 2026. Also see WHO, African Region, “Scaling up to stay ahead of the Ebola Outbreak,” September 30, 2026.

¹³⁸ Africa CDC, “G20+ Mobilizes US\$2.9 Billion for Ebola Response as Africa CDC Calls for Full Transparency and Traceability of Every Dollar,” September 24, 2026 (hereafter Africa CDC, September 24, 2026).

¹³⁹ Africa CDC, September 24, 2026.

China (\$18 million).¹⁴⁰ Africa CDC officials have emphasized the importance of financial pledges translating to delivery, stating that the organization work to strengthen its financial tracking mechanisms to “ensure greater visibility over commitments, disbursements, expenditures, implementation locations and results.”¹⁴¹

What is the current status of vaccines and therapeutics? How is the United States participating in such efforts?

As mentioned previously, there are no specific vaccines and treatments for Ebola disease caused by Bundibugyo virus. There are some authorized or approved vaccines and treatments that target the Zaire ebolavirus—the virus that drove previous large-scale Ebola outbreaks—and as of August 27, 2026, WHO has begun an emergency clinical trial of Ervebo, a Zaire ebolavirus vaccine, among health care workers in the DRC to determine whether the vaccine is effective against Bundibugyo virus in humans, as mentioned above.¹⁴² The following discusses efforts to develop and test new vaccines and treatments for Bundibugyo virus.

Vaccines

There are currently six Bundibugyo vaccine candidates in development, funded primarily by CEPI with financial contributions from European governments and the United States, and separately with funding from BARDA.¹⁴³

CEPI is a global partnership to develop vaccines, therapeutics, and other innovations to protect against epidemic threats. As of August 27, 2026, CEPI has funded research and clinical trials into five different vaccine candidates that use different vaccine technologies, with the goal that at least two of the candidates prove safe and effective in protecting against Bundibugyo virus.¹⁴⁴ CEPI is also funding some research on whether existing Zaire ebolavirus vaccines could provide protection against Bundibugyo virus to generate additional evidence in conjunction with the WHO clinical trial.¹⁴⁵

CEPI reports that its Bundibugyo virus vaccine development program is funded by the European Commission, the Department of State, and CEPI’s existing core funding.¹⁴⁶ In June 2026, the

¹⁴⁰ Africa CDC, September 24, 2026.

¹⁴¹ WHO, “WHO Director-General’s opening remarks at the media briefing,” September 16, 2026, and Africa CDC, September 24, 2026.

¹⁴² See citations in section on “What is Ebola disease and how does it spread?” and WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” last updated August 28, 2026, <https://www.who.int/emergencies/disease-outbreak-news/item/2026-DON616>.

¹⁴³ Coalition for Epidemic Preparedness Innovations (CEPI), “African Bundibugyo Ebolavirus Vaccine Candidate to be Advanced to Clinical Trials,” press release, August 26, 2026; and International Pandemic Preparedness Secretariat, *Ebola (Bundibugyo) Day 100 – Tracking Progress on Medical Countermeasures*, August 25, 2026. Of the five CEPI-funded candidates, BARDA is co-funding one. BARDA is also funding another candidate separately from CEPI.

¹⁴⁴ CEPI, “African Bundibugyo Ebolavirus Vaccine Candidate to be Advanced to Clinical Trials,” press release, August 26, 2026; CEPI, “Moderna’s Bundibugyo ebolavirus vaccine candidate enters first human trial, backed by CEPI,” August 3, 2026; CEPI, “Three vaccine designs in the race to contain Bundibugyo Ebola,” press release, June 4, 2026.

¹⁴⁵ CEPI, “New studies to test whether existing Ebola vaccines can generate immune responses to Bundibugyo ebolavirus,” press release, August 19, 2026.

¹⁴⁶ CEPI, “African Bundibugyo Ebolavirus Vaccine Candidate to be Advanced to Clinical Trials,” press release, August 26, 2026.

Department of State committed \$50 million to CEPI for vaccine development efforts.¹⁴⁷ As of September 4, CEPI has reported a funding shortfall of \$128 million of its \$270 million goal for its Bundibugyo R&D program.¹⁴⁸

BARDA has separately funded Bundibugyo vaccine development and reports coordinating with CEPI in these efforts.¹⁴⁹ BARDA, based within HHS's ASPR, has a specific mission to develop medical products to protect against public health threats.¹⁵⁰ As of June 9, 2026, BARDA had awarded \$33 million in additional funding under an existing contract with Public Health Vaccines, LLC, for developing vaccines against ebolaviruses, to initiate development of two Bundibugyo vaccine candidates. One of these candidates is also supported by CEPI. This BARDA-CEPI funded vaccine candidate originated from research conducted by scientists in the U.S. National Institutes of Health (NIH)'s National Institute of Allergy and Infectious Diseases (NIAID), a leading federal institute for vaccine and infectious disease research.¹⁵¹

In a June 26, 2026, press release, ASPR and BARDA announced a request for proposals to fund development of investigational Bundibugyo vaccine candidates, and it remains to be seen if BARDA will support additional vaccine candidates.¹⁵² NIAID also reports supporting early-stage research that could lead to potential Bundibugyo vaccine candidates.¹⁵³

All of the Bundibugyo virus vaccine development efforts are still in the early stages of R&D, though the pace of vaccine development has proceeded faster than in prior Ebola outbreaks.¹⁵⁴ The vaccine candidates that have progressed furthest in the development pipeline are still in early-stage Phase 1 clinical trials where they are tested in small numbers of people for initial evidence on safety and immune response.¹⁵⁵ If the vaccine candidates progress beyond Phase 1, the vaccines may be incorporated into WHO emergency clinical trials in the Bundibugyo outbreak area.¹⁵⁶

¹⁴⁷ CEPI, "U.S. Department of State commits \$50 million to CEPI to fast-track Bundibugyo virus medical countermeasures," June 11, 2026.

¹⁴⁸ Max Bayer, "A global health cash crunch could jeopardize the race for new Ebola vaccines," *Endpoint News*, September 4, 2026; and CEPI, "Outbreak emergency response: R&D funding urgently needed for Bundibugyo countermeasures," August 2026.

¹⁴⁹ ASPR, "ASPR Supports Global Ebola Response While Strengthening U.S. Preparedness," June 26, 2026.

¹⁵⁰ Biomedical Advanced Research and Development Authority (BARDA), "BARDA."

¹⁵¹ Public Health Vaccines, LLC, "Public Health Vaccines Awarded Option from BARDA to Further the Development of Vaccines Against Sudan Ebolavirus and Bundibugyo Ebolavirus," press release, June 9, 2026.

¹⁵² ASPR, "ASPR Supports Global Ebola Response While Strengthening U.S. Preparedness," June 26, 2026. For the request for proposals, see Rapid Response Partnership Vehicle (RRPV), "Bundibugyo virus Outbreak Response Vaccines (BundiVx)," June 17, 2026.

¹⁵³ National Institute of Allergy and Infectious Diseases (NIAID), "NIAID's Support of the Bundibugyo Virus Disease Outbreak Response," last updated July 7, 2026, <https://www.niaid.nih.gov/diseases-conditions/niaids-support-bundibugyo-virus-disease-outbreak-response>.

¹⁵⁴ Mona Nemer and Yenew Kebede Tebeje, "A Hundred Days into an Ebola Outbreak, the World Moved Fast. But the Gaps that Remain Should Worry Us All," *International Pandemic Preparedness Secretariat*, August 25, 2026.

¹⁵⁵ International Pandemic Preparedness Secretariat, *Ebola (Bundibugyo) Day 100 – Tracking Progress on Medical Countermeasures*, August 25, 2026; CEPI, "African Bundibugyo Ebolavirus Vaccine Candidate to be Advanced to Clinical Trials," press release, August 26, 2026; CEPI, "Moderna's Bundibugyo ebolavirus vaccine candidate enters first human trial, backed by CEPI," August 3, 2026; and CEPI, "CEPI-backed Bundibugyo ebolavirus vaccine trial launched by Oxford Vaccine Group," press release, July 12, 2026.

¹⁵⁶ WHO, Third Meeting of the WHO Technical Advisory Group on Candidate vaccine prioritization (TAG-CVP) for Bundibugyo virus disease outbreak response, July 31, 2026, <https://iris.who.int/server/api/core/bitstreams/591820e7-e23e-4ff4-bfee-b4958651e681/content>.

Therapeutics

There are two ongoing large-scale (Phase 3) clinical trials to test potential treatments for Ebola disease caused by Bundibugyo virus, as explained below.¹⁵⁷ The U.S. government and a U.S.-based pharmaceutical company have donated certain drugs for use in these clinical trials and for emergency or compassionate use in the DRC, meaning the drugs are provided to patients with life-threatening disease who have no other treatment options and cannot participate in a clinical trial.¹⁵⁸

The Platform Adaptive Randomized Trial for New and Repurposed Filovirus Treatments (PARTNERS) trial, sponsored by WHO with research support from many international academic partners and nongovernmental organizations, is testing several different possible treatments for Ebola disease caused by Bundibugyo virus.¹⁵⁹ As of August 28, 2026, the PARTNERS trial had enrolled 250 patients—the fastest-ever clinical trial for Ebola treatments, both in time to launch and pace of recruitment.¹⁶⁰ ASPR has transferred doses of one drug studied in this trial, MBP134, for research and compassionate use in the DRC.¹⁶¹ The NIAID had previously funded early laboratory research on this drug.¹⁶²

A different clinical trial, the EBO-PEP trial, is testing Gilead’s antiviral drug, obeldesivir, as a post-exposure prophylaxis for Ebola, meaning that the drug may prevent onset of disease after a person was exposed to the virus. The trial is led by the National Institute for Biomedical Research in Kinshasa, DRC, with research support from a French government research agency along with other nongovernmental organizations and humanitarian partners.¹⁶³ The European Commission, the Africa CDC, and other African governments have made financial contributions to the trial.¹⁶⁴

The U.S. NIAID also reports early-stage research (before clinical trials) on potential therapeutic candidates for Ebola disease caused by Bundibugyo virus.¹⁶⁵

¹⁵⁷ International Pandemic Preparedness Secretariat, *Ebola (Bundibugyo) Day 100 – Tracking Progress on Medical Countermeasures*, August 25, 2026.

¹⁵⁸ Gilead, “Gilead Mobilizes Rapid Donation of Remdesivir to Support Ebola Response, Building on Decade-Long Commitment to Outbreak Relief,” press release, June 12, 2026, <https://www.gilead.com/news/news-details/2026/gilead-mobilizes-rapid-donation-of-remdesivir-to-support-ebola-response-building-on-decade-long-commitment-to-outbreak-relief>; and ASPR, “ASPR Supports Global Ebola Response While Strengthening U.S. Preparedness,” June 26, 2026. For background on emergency or compassionate use of unproven therapeutics in an outbreak, see WHO, “Emergency Use of Unproven Clinical Interventions Outside Clinical Trials: Ethical Considerations,” March 25, 2025.

¹⁵⁹ WHO, “Patient enrolment begins in a scientific trial to identify the first effective treatments for Bundibugyo virus disease,” press release, July 2, 2026; and International Pandemic Preparedness Secretariat, *Ebola (Bundibugyo) Day 100 – Tracking Progress on Medical Countermeasures*, August 25, 2026.

¹⁶⁰ WHO, “Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo,” August 28, 2026, and International Pandemic Preparedness Secretariat, *Ebola (Bundibugyo) Day 100 – Tracking Progress on Medical Countermeasures*, August 25, 2026.

¹⁶¹ ASPR, “ASPR Supports Global Ebola Response While Strengthening U.S. Preparedness,” June 26, 2026.

¹⁶² NIAID, “NIAID’s Support of the Bundibugyo Virus Disease Outbreak Response,” last updated July 7, 2026, <https://www.niaid.nih.gov/diseases-conditions/niaids-support-bundibugyo-virus-disease-outbreak-response>.

¹⁶³ ANRS Emerging Infectious Diseases, “Bundibugyo Ebola virus outbreak: Launch of the EBO-PEP Trial to Protect High-Risk Contacts,” July 14, 2026; and Stephanie Soucheray, “Gilead launches Ebola antiviral trial in DR Congo as cases near 2,000,” *Center for Infectious Disease Research and Policy*, July 14, 2026.

¹⁶⁴ ReliefWeb, “Bundibugyo Ebola Virus Outbreak: Launch of the EBO-PEP Trial to Protect High-Risk Contacts,” press release, July 14, 2026, <https://reliefweb.int/report/democratic-republic-congo/bundibugyo-ebola-virus-outbreak-launch-ebo-pep-trial-protect-high-risk-contacts>.

¹⁶⁵ NIAID, “NIAID’s Support of the Bundibugyo Virus Disease Outbreak Response,” last updated July 7, 2026, <https://www.niaid.nih.gov/diseases-conditions/niaids-support-bundibugyo-virus-disease-outbreak-response>.

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